NCCP 00236 · VERSION 3

Your treatment, explained

NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy. · NCCP 00236 v3 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Vismodegib for advanced basal cell skin cancer

Reference calendar for daily medicine and reviews

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · ema-erivedge

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
The overall treatment plan
Your 28-day timeline
Baseline and cycle-specific checks
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UNDERSTAND YOUR TREATMENT

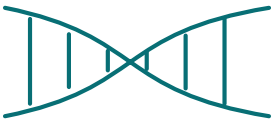
Medicines in this regimen

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

ORAL TARGETED CAPSULE

Vismodegib

Erivedge



Blocks the Hedgehog growth pathway

Vismodegib blocks a protein called Smoothened in the Hedgehog signalling pathway. This pathway can drive basal cell cancer growth. Blocking it can also affect normal hair follicles, taste and muscle function, which helps explain hair loss, altered taste and cramps. This treatment is different from a cream applied to a skin lesion.

Daily capsule → Hedgehog signal blocked → Cancer growth may slow

What to watch for

Keep four-week blood tests and the pregnancy-prevention programme. Report cramps, loss of strength, reduced intake, weight loss and severe skin symptoms.

Sources: nccp · nccn-patient · ema-erivedge

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Energy / daily activity

Vismodegib

Tiredness can affect daily tasks. Tell the team if it worsens or comes with dizziness, reduced eating or other new symptoms.

3 Taste / eating

Vismodegib

Taste may change or disappear, making food less appealing. Report reduced intake early and ask for dietitian help.

5 Muscles / strength

Vismodegib

Cramps and muscle pain are common concerns. Severe pain, weakness or dark urine needs urgent advice; do not add supplements or

7 Appetite / weight

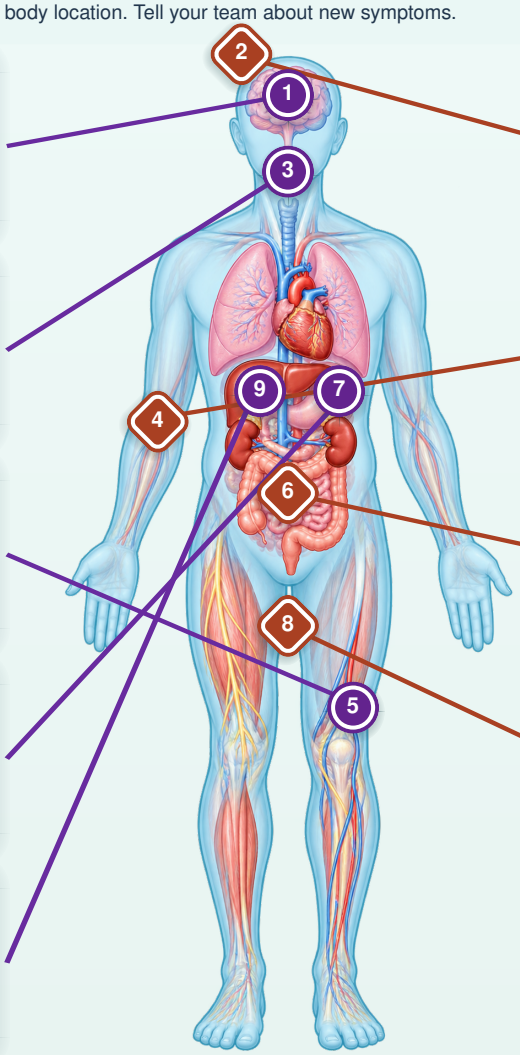
Vismodegib

Taste changes and poor appetite can cause weight loss. Keep a practical eating plan and tell the team about loose clothes or falling weight.

9 Liver

Vismodegib

Report yellow skin or eyes, dark urine, unusual itching or upper-abdominal pain. Keep the planned liver blood tests; a symptom needs assessment rather than self-diagnosis.



2 Hair / scalp

Vismodegib

Hair may thin or fall out. Ask about scalp care, sun protection and practical support; the timing of recovery varies.

4 Skin

Vismodegib

A rash with blistering, peeling, mouth sores or fever needs urgent assessment. Keep scheduled skin reviews and report new lesions.

6 Stomach / bowels

Vismodegib

Nausea, diarrhoea or constipation may occur. Call if symptoms persist, you cannot drink or urine becomes reduced.

8 Pregnancy / fertility

Vismodegib

A strict pregnancy-prevention programme applies to women and men, with restrictions continuing after treatment. Report possible exposure immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

The overall treatment plan

DAILY MEDICINES

Continuous oral treatment

No preset cycle total | **Vismodegib daily; four-week safety review. No routine treatment-free week.**

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00236 v3: current regimen source · NCCN Guidelines for Patients: Basal Cell Skin Cancer, 2026 - supplied education reference · EMA Erivedge product information, updated 5 November 2025

THE WHOLE TREATMENT JOURNEY

Vismodegib for advanced basal cell skin cancer: treatment and checks

NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy.

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00236	Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.	Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Cancer, skin and daily function NCCP 00236	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Keep follow-up for the treated lesion and for other skin cancers. Examination and any imaging are tailored to the disease and treatment; locally advanced disease needs an individual plan. Continuing precautions after vismodegib are part of follow-up even when the cancer medicine has stopped.
Medicines and continuing precautions NCCP 00236	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

What each check is for

1

Continuous oral treatment

Daily medicines

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00236

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily oral treatment	Days 1–28	Vismodegib daily; four-week safety review. No routine treatment-free week.

Treatment or check	Days in this window	What happens
Safety reviews as scheduled	Days 1–28	Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.
Symptoms and support	Days 1–28	Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

Timing note 1: Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Timing note 2: If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

Timing note 3: The formal pregnancy-prevention programme is essential; restrictions continue after stopping.

Timing note 4: A four-week review calendar does not create a routine drug holiday.

Timing note 5: Routine CPK testing is not specified by the cited NCCP schedule; muscle symptoms need individual assessment.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00236 v3: current regimen source ↗ · EMA Erivedge product information, updated 5 November 2025 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–28: Every day in the reference calendar

Vismodegib daily; four-week safety review. No routine treatment-free week. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Swallow the prescribed capsule whole with about 200 mL of water, with or without food, at the same time each day. Do not open, crush or chew it. Keep it in its labelled pack, away from children, and return unused capsules to the pharmacy.

STEP 2

Missed, vomited or held treatment

If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

STEP 3

Appointments and supply

Arrange dates, medicine supply, transport and any needed interpreter or carer support. Ask whether bloods are taken in the oncology unit or elsewhere and how the result reaches the prescriber.

STEP 4

Paper copies and contact information

Keep this complete handbook, the three-page summary and the wallet card together. The QR code is optional. Ask the team to add your actual contacts and print a revised plan when treatment changes.

Sources: NCCP 00236 v3: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Formal pregnancy-prevention programme

The programme materials, supervised tests, contraceptive methods and after-treatment restrictions must be agreed and documented before the first dose; they apply even during a treatment pause.

2 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

3 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

4 Pregnancy and fertility precautions

Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

5 Eating, activity and practical help

Report falling intake, weight loss, pain or fatigue early. The team can tailor nutrition, activity, wound, psychological and practical support to your needs.

Sources: nccp · nccn-patient · ema-erivedge

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

Confirm the diagnosis and extent

Keep the biopsy result, original site, dates and details of previous treatment. Basal cell carcinoma usually remains local, but some tumours become difficult to remove or rarely spread. Your team should explain whether the present problem is locally advanced disease or metastatic disease.

Sources: nccn-patient

Why tablets are being considered

The cited NCCP vismodegib pathway concerns symptomatic metastatic BCC, or locally advanced BCC unsuitable for surgery or radiotherapy. Ask what makes those local treatments unsuitable in your case and what benefit is realistic. A systemic capsule is different from a cream for a small superficial lesion.

Sources: nccn-patient

Record earlier local treatment

Record operations, margin results, reconstruction, radiotherapy and any previous BCC medicines. The effect on sight, movement, pain, wound healing and appearance may influence choices. More than one specialist may be needed to agree the sequence and practical support.

Sources: nccn-patient

Inherited conditions and multiple cancers

Tell the team if you have had many BCCs at a young age or an established inherited condition such as Gorlin syndrome. This does not mean everyone with BCC needs genetic testing. Ask whether a specialist skin-surveillance or genetic advice plan is appropriate for your own history.

Sources: nccn-patient

A visible lesion is only one part of assessment

Changes in a lesion, pain or a wound may be useful to track, but they do not replace examination and any agreed imaging. Report new skin lesions separately. If you use photographs, agree how and when the clinical team will compare them and how you will keep them private.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and the overall plan

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

The schedule in this setting

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Sources: nccp

How the treatment works

Vismodegib blocks a protein called Smoothened in the Hedgehog signalling pathway. This pathway can drive basal cell cancer growth. Blocking it can also affect normal hair follicles, taste and muscle function, which helps explain hair loss, altered taste and cramps. This treatment is different from a cream applied to a skin lesion.

Sources: nccp

The role of earlier or later treatment

Record surgery, radiotherapy and other anticancer medicines separately, including dates and the specialist responsible. This schedule does not automatically include or authorise another treatment. Ask whether any overlap changes tests, symptoms to report or the timing of appointments.

Sources: nccn-patient

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Sources: nccp

TREATMENT REFERENCE · 03

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Swallow the prescribed capsule whole with about 200 mL of water, with or without food, at the same time each day. Do not open, crush or chew it. Keep it in its labelled pack, away from children, and return unused capsules to the pharmacy.

Sources: nccp

Missed, vomited or held treatment

If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

Sources: nccp

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Sources: nccp

Practical preparation

Arrange medicine supply, transport, interpreter or carer support before it is needed. Bring the alert card, symptom record and questions. Ask which appointments require someone to accompany you and how to obtain urgent advice outside normal hours.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.

Sources: nccp

During treatment

Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.

Sources: nccp

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Sources: nccp

After a pause or finish

Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.

Sources: nccp

Getting the result

For each test, record its purpose, due date, result and action. Agree how results reach you and who checks a result from another hospital. If an appointment or result has not arrived, contact the named service rather than assuming no news means no action is needed.

Sources: nccn-patient

TREATMENT REFERENCE · 05

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Sources: nccp

When to call

Call urgently for blistering or peeling skin, mouth sores with rash or fever, severe muscle pain or weakness, or dark urine. Seek immediate advice about suspected pregnancy or exposure. Report taste change, reduced eating and weight loss early so support can start.

Sources: nccp

Eating, drinking and activity

Tell the team early about nausea, taste change, a sore mouth, diarrhoea or falling weight. Ask for an individual fluid and food plan if heart, kidney or other conditions limit intake. Gentle activity and planned rest can help maintain function; adjust to symptoms and ask for support when fatigue limits daily life.

Sources: nccn-patient

A held dose and a new phase

If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

Sources: nccp

TREATMENT REFERENCE · 06

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Use shade, protective clothing, a hat and broad-spectrum sunscreen of at least SPF 30, and avoid tanning beds. Reapply sunscreen according to its instructions. Protection matters even when it is cloudy; choose a practical approach that lets you remain active safely.

Sources: nccn-patient

Know the pattern of your skin

Ask how to examine your skin, scars and relevant lymph-node areas and how often your own team recommends doing so. A mirror or a trusted helper can make hard-to-see areas easier to check. Report a new or changing lesion, a non-healing sore or a new lump rather than waiting for a routine visit.

Sources: nccn-patient

Photographs are optional

If useful, keep a dated photograph with consistent lighting and a size reference, following your team's advice. Photographs are optional and do not diagnose a cancer or replace examination. Keep them privately; this printable passport does not require you to upload images or other personal health information.

Sources: nccn-patient

Wounds, nails and sensitive skin

Use the dressings and skin-care plan recommended for your lesion and treatment. Ask before applying new creams or remedies to an open or treated area. Worsening pain, discharge, redness, bleeding or loss of function needs advice; routine skin-care instructions are not a substitute for assessment.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

Sources: nccp · ema-erivedge

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Sources: nccp · ema-erivedge

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Sources: nccp · ema-erivedge

Testing and medicine supply

When pregnancy is possible, the initial prescription and dispensing must occur by day seven, counting the pregnancy-test date as day one. Supplies are limited to 28 days and need a renewed prescription. Ask the team to coordinate supervised monthly testing and supply so that these safeguards are maintained.

Sources: ema-erivedge

TREATMENT REFERENCE · 08

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what evidence applies to your diagnosis, stage, biomarker and treatment setting, what outcome the team hopes to improve, and what may happen without this treatment. A trial average cannot predict an individual result; a benefit measured in one setting should not be transferred to another.

Sources: nccn-patient

Alternatives and review points

Discuss other suitable treatments, clinical trials if relevant, local treatment options and symptom-focused care. Ask when the decision will be reviewed and what would prompt a change. Supportive and palliative care can help with symptoms and quality of life alongside anticancer treatment.

Sources: nccn-patient

Help with daily life

Tell the team about work, transport, caring responsibilities, cost concerns, body image, anxiety or sleep. Ask which nurse, social worker, dietitian, rehabilitation professional or support organisation can help. Use resources relevant to your country and service; a US guide may describe different access or funding.

Sources: nccn-patient

Shared understanding

Ask someone on the team to check that you can explain the medicine schedule, next tests and urgent-contact route in your own words. An interpreter, large-print copy or a trusted supporter can help; none requires you to use a smartphone.

Sources: nccn-patient

TREATMENT REFERENCE · 09

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Keep follow-up for the treated lesion and for other skin cancers. Examination and any imaging are tailored to the disease and treatment; locally advanced disease needs an individual plan. Continuing precautions after vismodegib are part of follow-up even when the cancer medicine has stopped.

Sources: nccn-patient

Continuing medicine precautions

Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.

Sources: nccp

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

Sources: nccp

The complete paper version

This handbook can be used without a phone: keep the schedule, symptom advice, medicine list, tests and contacts together. The QR code is an optional route to the matching regimen information. Ask the team to print updates when your plan changes and keep the latest dated copy.

Sources: nccn-patient

Evidence in advanced basal cell carcinoma

ERIVANCE studied vismodegib without a comparison treatment. At an analysis 21 months after the last person joined the study, independent reviewers found a confirmed response in 11 of 33 people with metastatic BCC (33.3%; 95% CI 19.2-51.8%) and 30 of 63 with locally advanced BCC (47.6%; 95% CI 35.5-60.6%). The analysis date does not mean everyone took treatment for 21 months.

A response means that the study's cancer-response criteria were met. The assessment combined scans for metastatic disease and lesion measurements, ulcer healing and biopsy information for locally advanced disease. Response is not the same as cure and does not prove longer survival compared with another treatment.

Sources: ema-erivedge · erivance-2012 · erivance-2015

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of my treatment?

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

Is there a fixed number of cycles?

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

What happens if treatment is held?

If a dose is missed, skip it and take the next dose at the usual time. Do not double up. If you vomit after taking it, ask the oncology team or pharmacist before taking more; do not assume that the dose needs replacing.

When are my next tests?

Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00236 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00236 v3: current regimen source
- NCCN Guidelines for Patients: Basal Cell Skin Cancer, 2026 - supplied education reference
- EMA Erivedge product information, updated 5 November 2025

- ERIVANCE original study: Sekulic and colleagues, NEJM 2012
- ERIVANCE independent-review follow-up: Sekulic and colleagues, JAAD 2015

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6725/236_v3_Vismodegib.pdf

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